

Locum Tenens Onboarding Checklist

Everything your facility needs to onboard a temporary provider safely and efficiently

- ✓ Pre-arrival credentialing and privileging checklist (22 items)
- ✓ Day-one orientation protocol
- ✓ EMR access and clinical workflow setup
- ✓ Performance monitoring and assignment management

Pre-Arrival Checklist (30-90 Days Before Start)

Credentialing & Privileging

- ☐ Receive complete credentialing file from agency (CV, licenses, certifications, NPDB, references)
- ☐ Verify state medical license is active and unrestricted for your state
- ☐ Verify board certification status via ABMS Certification Matters
- ☐ Run independent OIG/SAM exclusion check (do NOT rely solely on agency)
- ☐ Verify DEA registration is active for your state
- ☐ Complete internal credentialing review per medical staff bylaws
- ☐ Present to Credentials Committee for temporary or locum privileges
- ☐ Obtain Medical Executive Committee (MEC) approval
- ☐ Board of Directors approval (if required by bylaws)
- ☐ Issue temporary privilege letter with specific clinical privileges delineated

Administrative Setup

- ☐ Execute locum tenens contract with agency (or direct contract with provider)
- ☐ Obtain Certificate of Insurance from agency — confirm occurrence-based coverage
- ☐ Add provider to malpractice insurance (if facility provides coverage)
- ☐ Assign NPI/Tax ID for billing under your facility's group
- ☐ Enroll provider in facility's EMR system with appropriate access level
- ☐ Set up dictation/transcription access
- ☐ Order ID badge with photo, name, "Locum Tenens" designation, and department

- ☐ Assign parking, call room, locker, mailbox
- ☐ Set up paging/communication system access
- ☐ Assign medical staff office contact as point person for the locum

CMS Requirement

Temporary privileges must be granted through your Medical Staff process per CMS Conditions of Participation (42 CFR §482.22). The CMO or department chief cannot unilaterally grant privileges. Follow your bylaws exactly — shortcuts create survey exposure.

Day-One Orientation Protocol

First-Day Orientation Checklist for Locum Providers

Morning Orientation

- ☐ Meet department chief and medical staff office representative
- ☐ Facility tour: ED, OR, ICU, floors, lab, radiology, pharmacy, cafeteria
- ☐ Review emergency codes and procedures (Code Blue, Rapid Response, fire, active shooter)
- ☐ Review call schedule and coverage expectations
- ☐ Review patient hand-off protocol (I-PASS, SBAR, or facility-specific format)
- ☐ Review admission/discharge protocols and case management contacts
- ☐ Review pharmacy formulary and prescribing restrictions
- ☐ Review restraint, blood transfusion, and consent policies
- ☐ Exchange contact information with nursing leadership, department staff, and hospitalist team

EMR Training

- ☐ Login credentials issued and tested
- ☐ Provider-specific EMR orientation (note templates, order sets, documentation requirements)
- ☐ CPOE (Computerized Physician Order Entry) walkthrough
- ☐ E-prescribing setup and verification
- ☐ Demonstrate where to find lab results, imaging, consult notes
- ☐ Review required documentation for billing compliance (E&M levels, procedure notes)
- ☐ Assign EMR "super user" buddy for first week questions

Compliance Sign-Offs

- ☐ HIPAA acknowledgment signed
- ☐ Facility confidentiality agreement signed
- ☐ Drug-free workplace acknowledgment signed
- ☐ Conflict of interest disclosure completed
- ☐ Review of facility's clinical documentation integrity (CDI) program
- ☐ Review AMA and patient rights policies

EMR Buddy System

Assign an experienced permanent provider or clinical informaticist as the locum's EMR buddy for the first 3-5 days. EMR-related productivity loss is the #1 complaint from both locums and facilities. A buddy system reduces ramp-up time by 40-60%.

Clinical Integration Checklist

Integrating the Locum Into Your Clinical Workflow

Week 1 — Supervised Ramp-Up

- ☐ Department chief or designee reviews first 5-10 charts for documentation quality
- ☐ Confirm locum is following facility-specific order sets and clinical pathways
- ☐ Verify billing/coding documentation meets facility standards (E&M levels correct)
- ☐ Check that handoffs to incoming providers are complete (I-PASS, written sign-out)
- ☐ Confirm dictation/transcription workflow is functioning
- ☐ Address any EMR or workflow questions before end of first week

Ongoing

- ☐ Include locum in department meetings and relevant M&M conferences
- ☐ Assign locum a departmental buddy for clinical questions (not just EMR)
- ☐ Monitor patient satisfaction scores for locum-seen patients
- ☐ Track locum's clinical metrics (length of stay, readmission, throughput) alongside permanent staff
- ☐ Conduct biweekly check-ins with locum and department chief

Productivity Expectation

Most locum providers reach 80% of a permanent provider's productivity by week 2, and full productivity by week 3-4. Factor this ramp-up into your budget — the first week is an investment in orientation, not full clinical output.

Agency Communication & Issue Resolution

Managing the Agency Relationship During the Assignment

Contact Protocol

Issue	First Contact	Escalation
Clinical performance concern	Department Chief → CMO	Agency account manager
Scheduling/availability conflict	Locum directly	Agency account manager
Billing/invoice dispute	Facility AP → Agency billing	Agency VP/Director
Housing or travel problem	Agency account manager	Agency operations director
Behavioral/professionalism issue	CMO → Agency immediately	Agency clinical leadership
Early termination (either side)	Agency account manager	Facility legal + Agency legal
Conversion to permanent hire	Agency account manager	Negotiate conversion fee per contract

Key Contract Provisions to Monitor

- ☐ Confirm invoice matches contracted bill rate (check for unauthorized rate increases)
- ☐ Verify overtime is calculated per contract terms (> 40 hrs/week or > 12 hrs/day)
- ☐ Confirm call coverage is billed at the agreed rate
- ☐ Review monthly invoices for charges not in the original agreement
- ☐ Track actual days/hours worked against projections for budget variance

Early Termination

Most locum contracts allow either party to terminate with 14-30 days notice. However, some contracts include penalties for early facility-side cancellation. Review your contract's termination clause before signing — not after a problem arises.

Performance Monitoring Dashboard

Tracking Locum Provider Performance — Key Metrics

Metric	Target	Week 1	Week 2	Week 3	Week 4
Patient volume (per shift)	≥80% of permanent staff	_____	_____	_____	_____
Average length of stay	Within department avg	_____	_____	_____	_____
30-day readmission rate	≤ department avg	_____	_____	_____	_____
Documentation completion (24 hrs)	100%	_____	_____	_____	_____
Patient satisfaction (if available)	≥ 4.0/5.0	_____	_____	_____	_____
Nursing/staff feedback	Positive	_____	_____	_____	_____
Handoff quality (peer assessment)	Complete	_____	_____	_____	_____
Coding accuracy (chart audit)	≥ 95%	_____	_____	_____	_____

End-of-Assignment Checklist

- ☐ Final chart review — all notes complete and signed
- ☐ Return ID badge, parking pass, keys, pager, and any facility equipment
- ☐ Deactivate EMR access and all system credentials on last day
- ☐ Complete facility evaluation of provider (share with agency)
- ☐ Complete provider evaluation of facility (helps attract future locums)
- ☐ Confirm final invoice reconciliation with agency
- ☐ Address malpractice tail coverage (if claims-made policy)

☐ Discuss potential for return assignment or permanent conversion

Cost-Benefit Analysis: Locum vs. Vacancy

Side-by-side comparison

Factor	Locum Coverage	Leave Position Vacant
Locum cost (40 hrs/wk × 13 wks × \$225/hr)	\$117,000	\$0
Lost revenue from unfilled shifts	\$0	\$150,000–\$300,000
Diversion/transfer costs	\$0	\$25,000–\$75,000
Remaining staff overtime/burnout	Minimal	Significant
Patient satisfaction impact	Maintained	Decline
Permanent staff turnover risk	Low	High (burnout-driven)
Net financial impact	-\$117,000 cost	-\$175,000 to -\$375,000 lost revenue

A vacant hospitalist position costs facilities an estimated **\$150,000–\$300,000 per quarter** in lost downstream revenue (admissions, procedures, ancillary services). Locum coverage at \$117,000/quarter is typically revenue-positive when downstream economics are considered.

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